



Nuclear Cardiology Department

Patient Profile

M.R.No. : [redacted]
Name : [redacted]
Age : 50Yr(s) 1Mth(s) 24Day(s)
Sex : Male
Address : VIII KOT BHERO PO DO
Personal Phone : [redacted]

Departmental Ref # : [redacted]
In-house Consultant :
O/S Referring Physician :
Order by : Invalid MRNO
Performed Date : 23 JAN 2012 05:53:25 PM
Signed Date : 30 JAN 2012 09:44:57 AM

THALLIUM-201 SCAN -- STRESS (PHARMACOLOGICAL) - DELAYED REPORT

History (1) Age 49 year old gentleman with a BMI of 24. A known patient of (2) DVCAD (LAD and RCA). C/o chest pain, off and on. 2-D echo showed anteroapical hypokinesia.

Procedure Injection adenosine infusion (140 microgram / kg/min) over six minutes was given through an infusion pump. Injection thallium 201 (3mCi) was given (IV) at 3 minutes of infusion. Patient had no complains during or after the infusion. Multiple images (stress) were taken with a dual head SPECT camera immediately after infusion. Same images (delayed) were repeated after 3 hours. Thallium - 201 (1 mCi) reinjected 15 minutes before delayed images. No significant change was noted in the resting ECG during or after adenosine infusion. Resting ECG showed QS V2-V4. (3) ECG-Leads

(4) Heart Rate — Before infusion — 76 BPM ✓
Max achieved — 84 BPM ✓
(5) BP — Before infusion — 130/80 mmHg ✓
Max achieved — 120/70 mmHg ✓

Scan Finding: Myocardial perfusion post-adenosine hyperaemic stress images show dilated LV with some (6) tracer uptake in the lungs. There is diminished tracer uptake in the (7) apex, apical to mid anterior wall segments with no significant change on ECG gated delayed images. Resting LVEF is 33% with EDV of 132 ml and ESV of 88 ml. In cine mode, anteroapical area is severely hypokinetic with definite reduction in systolic wall thickening. (8) Trace Region

Impression: 1. A medium sized (3/10) LAD segments, non-viable, defect involving apex and apical to mid anterior wall segments. (9) defect in LAD ✓
2. Viable remaining LV myocardium. ✓
3. Resting LVEF is 33%. (10) LVEF is low

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COL MOHSIN SAEED SHAIKH

Comdt / Exec Dir
MAJ GEN SHAHAB NAQVI

CAUTION: Please carry this report with you, when travelling abroad, even many weeks after your scan, as the radiation from residual radiolotope in the body, may trigger sensitive radiation alarms, installed at international airports.

Fig. 2. Cardiac Patient's Report